

February 20, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology
Attention: Request for Information: HHS Health Sector AI RFI
Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

Submitted electronically to Regulations.gov

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Dr. Keane:

Premier appreciates the opportunity to submit comments to the Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) on the Request for Information (RFI) on artificial intelligence (AI) adoption in clinical care. Premier values the Administration's ongoing commitment to tech-enabling healthcare to reduce administrative burden while improving access to high-quality care. Premier has consistently advocated for a **responsible, innovation-friendly and transparent regulatory framework for AI in healthcare**, aligning with our longstanding commitment to improving patient outcomes and supporting clinicians through technology. Our policy positions emphasize the need for **clear regulatory guardrails, strong data privacy protections, robust workforce enablement and industry-aligned standards** for safe and effective AI adoption.

In our detailed comments below, Premier urges ASTP/ONC to:

- Support the harmonization of regulatory requirements across the Department of Health and Human Services (HHS) to reduce duplicative or conflicting regulatory burdens;
- Provide technical assistance and guidance to help private sector organizations understand and evaluate cybersecurity and privacy risks in acquiring and deploying AI technologies;
- Coordinate across HHS to incorporate hospital cybersecurity needs into the hospital preparedness program funding formula;
- Work with the Centers for Medicare and Medicaid Services (CMS) to reduce regulatory barriers to AI and software coverage and financing;
- Explore opportunities for transparency labeling of AI technology in healthcare, which should include outcomes-focused metrics such as accuracy, false positives, inference risks and recommended use/applications;
- Embed supply chain identifiers in ONC certification criteria and requirements for application programming interfaces (APIs);
- Publish cross-agency guidance endorsing open standards for supply chain data pipelines for AI tools; and
- Coordinate across HHS – particularly with the Food and Drug Administration (FDA) – to reinforce the Global Trade Identification Number (GTIN) as the standard product key in provider systems and item masters, allowing AI agents to link key information to provide timely, clinically-relevant insights.

I. BACKGROUND ON PREMIER INC.

Premier is a leading healthcare improvement company and national supply chain leader, uniting an alliance of 4,250 hospitals and approximately 365,000 continuum of care providers to transform healthcare. With integrated data and analytics, collaboratives, supply chain solutions, consulting and other services, Premier enables better care and outcomes at a lower cost.

A Malcolm Baldrige National Quality Award recipient, Premier plays a critical role in the rapidly evolving healthcare industry, collaborating with healthcare providers, manufacturers, distributors, government and other entities to co-develop long-term innovations that reinvent and improve the way healthcare is delivered nationwide. Headquartered in Charlotte, North Carolina, Premier is passionate about transforming American healthcare.

II. EARLY SUCCESSES IN CLINICAL DEPLOYMENT OF AI

Premier has partnered with leading healthcare providers and innovators to leverage AI in clinical workflows and improve patient outcomes. Early detection and intervention can improve patient outcomes and drastically reduce overall healthcare costs – for both patients and providers. Premier has demonstrated success in the following areas:

- The Premier team utilized artificial intelligence (AI), NLP and a data ontology designed to mine the unstructured narrative of clinicians' notes and pathology reports to identify patients for early Alzheimer's Disease [intervention](#).
- In oncology, Premier [partnered](#) with AstraZeneca and Clinithink and utilized Clinithink's CLiX NLP technology to identify patients with incidental pulmonary nodules (IPNs) to flag for intervention before potential lung cancer progression – with roughly 152,000 patients "caught" early.
- Premier worked with GE Healthcare and St. Luke's University Health Network to introduce a patient-centric care model for breast cancer diagnosis – with a goal of helping patients go from appointment to diagnosis and connection to a treatment plan in just 48 hours or less.

Premier sees particular promise for the use of AI in streamlining processes and expanding patient access in clinical trials:

- **Identifying trial participants:** One of the biggest challenges facing health systems that seek to participate in or enroll patients in clinical trials is identifying and enrolling patients in a timely manner. Delays in meeting trial enrollment targets and timelines can increase the cost of the trial. AI tools have the [ability](#) to analyze the extensive universe of data available to healthcare systems to identify patients that may be a match for clinical trials that are currently recruiting. This application of natural language processing systems can make developing new drugs less expensive and more efficient, while also improving patient and geographical diversity in trials to address generalizability.
- **Generating synthetic data:** AI, once trained on real-world data (RWD), has the capability to generate [synthetic data](#) and patient profiles that share characteristics with the target patient population for a clinical trial. This synthetic data can be used to simulate clinical trials to optimize trial designs, model the possible effects or range of results of a novel intervention, and predict the statistical significance and magnitude of effects or biases. Ultimately, synthetic patient data can help optimize trial design, improve safety and reduce cost for decentralized clinical trials. Further, synthetic control arms in clinical trials can help increase trial enrollment by easing patient fears that they will receive a placebo. ***To incentivize continued innovation, Premier encourages ASTP/ONC to work with the Food and Drug Administration (FDA) to promulgate clear guidance on the process for properly obtaining consent from patients for the use of their RWD to produce AI-generated synthetic control arms in clinical trials.***

III. BARRIERS TO PRIVATE SECTOR INNOVATION AND CLINICAL ADOPTION

Based on Premier's real-world experience building, deploying and supporting AI tools across supply chain management, drug manufacturing, clinical decision support and healthcare operations, a consistent set of structural, regulatory, operational and economic barriers continue to limit private sector innovation in AI and impede adoption in clinical settings.

1. **Lack of national privacy standards.** A significant barrier to the large-scale diffusion of innovative AI applications in healthcare is the lack of a single, comprehensive federal privacy law. A cohesive national

privacy framework is necessary to provide a reasonable level of consumer confidence that businesses will protect their data, sensitive or otherwise. The current patchwork, state-driven approach to privacy policy has resulted in inconsistent data privacy practices. Patients must be able to trust the processes for storage, handling and use of their data, particularly as patient data is increasingly used to train AI algorithms. ***Missing data from patients wary of data sharing could add to data siloing and perpetuate AI algorithmic biases, and ASTP/ONC should work with data stakeholders across HHS to proactively educate patients on these tradeoffs.***

Unharmonized and burdensome requirements are a rate-limiting step in unlocking value through AI innovation. A federal privacy law can empower innovators by placing clear, harmonized and common-sense guardrails around artificial intelligence tools. Healthcare privacy enforcement authorities are currently diffused across a wide swath of the federal government. The Health Insurance Portability and Accountability Act (HIPAA) only delegates privacy enforcement authority to the Department of Health and Human Services (HHS) Office of Civil Rights (OCR) for a narrow subset of covered entities. With the proliferation of a healthcare data ecosystem outside of the traditional payer-provider exchange, enforcement authority falls to the Federal Trade Commission, the Department of Justice and other agencies for a broad range of privacy- and data-related violations. There is significant, urgent need for a unified baseline set of enforcement priorities to create a harmonized enforcement authority for the portion of healthcare ecosystem outside of the bounds of HIPAA

Now, as state AI laws follow a similar path toward fragmentation, ***it is critical that ASTP/ONC and its regulatory partners either 1) offer directional federal guidance or 2) formally defer to the private sector to set AI security, transparency, safety and quality terms through the contracting process.*** Fragmented regulatory approaches across agencies and states impede adoption and increase compliance burdens. ***Premier strongly supports a OneHHS approach that harmonizes requirements across HHS operating divisions.***

2. **Workflow integration challenges.** The clinical workforce has experienced significant difficulty in integrating AI tools within existing clinical workflows, EHR systems and operational infrastructure. AI tools often must conform to complex and aging infrastructure, requiring expensive adaptation and reconfiguration. Many tools fail to accommodate workflow constraints faced by clinicians, leading to low adoption and poor usability.

Additionally, implementing AI can introduce new workforce demands and training needs, especially when tools alter established clinical processes. These constraints mean that even high-value AI products struggle to be adopted at the point of care. Premier has long supported the development of a clinical workforce trained to maximize the potential of AI and mitigate risks such as automation bias. Clinicians must understand when and how AI recommendations are generated, and they should be empowered, not replaced, by technology. ***Premier urges HHS to invest in education programs, establish best practices for human-AI collaboration and promote AI tools that measurably reduce administrative burden.***

3. **Uncertainty around liability, risk and governance.** Health care providers need clear information from the developers of AI tools to ensure safe and appropriate use. For example, in order to determine whether it is safe and appropriate to deploy an AI solution in a clinical setting, providers will need to know that a developer has undertaken efforts to ensure that an AI tool is accurate and safe for specific populations, whether it has gone through the FDA approval process and an explanation of how the AI model works.

Researchers and early adopters have identified concerns about AI tools in use in health care today that may embed bias or have other unintended clinical consequences. More transparency on the training data and algorithms used to create AI tools, as well as results from analyses and evaluations of AI tools, will support clinicians and other end-users in assessing whether and how to use these tools. Premier has previously supported the concept of model cards or other standardized transparency disclosures for high-risk AI deployment scenarios in clinical settings.

IV. RECOMMENDATIONS FOR REGULATORY AND PAYMENT POLICY CHANGES

Cybersecurity

In order to drive greater adoption of AI in clinical practice, clinicians and healthcare organizations need assurances that these novel solutions are cybersecure. Premier has firsthand experience with the challenges that provider organizations face evaluating the security and dependencies of novel AI solutions and AI-enabled software. As a leading traditional healthcare group purchasing organization (GPO), Premier works with several thousand healthcare suppliers and vendors, including medical device manufacturers and digital health companies, to contract with hospitals and health systems. Premier believes that securing healthcare's digital infrastructure requires operators to have visibility into vendor security practices. To that end, Premier requires digital health vendors to complete over 100 questions about security practices, independent third-party cybersecurity attestation or certification, company governance, and use of AI, among other details. Hospitals and health systems are then able to review this information before determining whether to offer that vendor a contract.

Premier's contracting process includes supporting healthcare operators with model contract language around cyber and AI liability and vulnerability disclosure and management. ***Premier encourages ASTP/ONC to provide additional technical assistance and guidance to private sector organizations that can better prepare vendors to collect and provide relevant AI data and practices, and can help operators to understand and evaluate the risks present during contracting.***

Additionally, the financial risk associated deploying novel technology also has a chilling effect on the adoption of AI in healthcare. Over the past six years, downtime from ransomware attacks has cost healthcare organizations an average of \$1.9 million per day, [according to one report](#). It is [estimated](#) that 92 percent of healthcare organizations have experienced at least one cyberattack in the past 12 months. ***Premier urges ASTP/ONC to work with the Assistant Secretary for Preparedness and Response (ASPR) to explicitly add cyberattacks as a component of the hospital preparedness program (HPP) funding formula to help make funding available for cyber resiliency.*** Premier also encourages HHS to work with federal partners, including cybersecurity coordinators like the Cybersecurity & Infrastructure Security Agency (CISA) and the Office of the National Cyber Director and security partners like the FBI, NSA, and Department of Defense, to understand the pressing, multi-dimensional threat that cybersecurity poses to America's healthcare delivery systems. These partnerships can foster a coordinated approach to cybersecurity in healthcare and provide insights on appropriate data sources for the HPP funding formula.

Data-driven financing pathways

Performance-based contracting arrangements – and data collected on AI and software performance – can help to decrease the go-to-market time and cost for new AI solutions, further extending America's lead in innovative technology. Through our experience as a leading healthcare GPO, Premier has a long track record of empowering health systems to embrace innovative technologies, increase market competition and reduce the cost of healthcare.

Premier also has firsthand experience facilitating performance-based contracts between healthcare vendors and providers to ensure the new technologies they purchase deliver the promised benefits and return on investment. Premier also regularly evaluates new technology and AI solutions for inclusion in Premier's digital health GPO contracts. Premier has learned valuable lessons from tackling the unique challenges posed by payment for software-as-a-service (SaaS) in healthcare. ***Premier urges ASTP/ONC to work with CMS to reduce regulatory barriers to AI and software approval and payment. HHS should explore ways to allow data collected from performance-based contracting arrangements to offer a pathway to CMS coverage and/or reimbursement decisions for novel digital health technologies and SaaS solutions.*** Hospitals have already entered performance-based contracts with innovative new healthcare suppliers, including AI technology and SaaS vendors. By making payment dependent on predefined outcomes, these contracts make it easier for hospitals to take a chance on new technologies. If the purported return on investment fails to materialize, hospitals can recoup initial costs.

Currently, the lack of reliable reimbursement for SaaS and digital health solutions means that providers must view new technologies as a "pay now, save later" proposition. However, tight margins often make it challenging for hospitals to invest up front in unproven technologies that may or may not result in savings down the road. For these

reasons, SaaS AI technologies face broad barriers to widespread use in healthcare. By incorporating information from standard performance-based contracts as part of the pathway to payment, CMS can encourage early adoption. ASTP/ONC should work with CMS to reduce regulatory barriers and open new pathways to sustainable financing for novel digital health technologies and AI solutions.

Transparency

Trust – among patients, providers, payers and suppliers – is critical to the development and deployment of AI tools in healthcare settings. To earn trust, AI tools must have an established standard of transparency. Some policy proposals, including [those proffered by ONC](#), suggest transparency can be achieved through a “nutrition label” model, which lists the sources and classes of data used to train the algorithm. Unfortunately, some versions of the “nutrition label” approach to AI transparency fail to acknowledge that when an AI tool is trained on a large, complex dataset, and is by design intended to evolve and learn, the initial static inputs captured by a label do not provide accurate insights into an ever-changing AI tool. Further, disclosure requirements around data inputs or algorithmic processes, if not carefully considered, could force AI developers to publicly disclose intellectual property or proprietary technology, which would stifle innovation.

Premier recommends that AI technology in healthcare should be held to a standardized, outcomes-focused set of metrics, such as accuracy, false positives, inference risks and recommended use/applications. Outcomes, rather than inputs, are where AI technologies hold potential to drive health or harm. Thus, Premier believes it is essential to focus transparency efforts on the accuracy, reliability and overall appropriateness of AI technology outputs in healthcare to ensure that the evolving tool does not produce harm. Premier has heard from multiple member hospitals that the lack of clear vendor information about the use of AI and associated liability actively deters them from purchasing or using AI tools. The lack of personnel and budget to collect information on data use, cybersecurity and liability terms from vendors exacerbates this issue for all but the biggest health systems.

Premier urges HHS to consider requiring AI developers and manufacturers to list the acceptable uses of new technologies in healthcare settings, which would provide much-needed guidance to clinicians and providers on safe and appropriate use cases. This approach could provide liability protection for the proper use of AI technology for the defined set of use cases where developers have established and reported the appropriate metrics for accuracy and reliability. Transparency about the intended use of AI tools would be the simplest way for regulators to incorporate AI governance into existing regulations. Health systems would be also able to incorporate this information into their own governance structures, putting internal policies in place to prevent misuse of AI in ways that could be detrimental to patient safety or experiences.

Such disclosure would give health systems and patients a complete picture of the safety and security of the AI technologies they use. Rather than limiting or delaying innovation, such guidelines would level the playing field between established market leaders and startups. **Alternatively, HHS could sanction the use of third-party certification organizations or existing market processes to address this challenge while reducing administrative burden. As a GPO, Premier already requires vendors and suppliers to submit information about uses of AI, data and cybersecurity certifications, and AI standards.** Such a market-driven approach would be flexible and adaptable, capable of adjusting to the latest developments in AI technology without requiring regulators to frequently revisit the framework.

Harmonization of Requirements for Rural Providers

Premier urges ASTP/ONC to explicitly align any forthcoming AI policies with the Rural Health Transformation (RHT) grants authorized under the One Big Beautiful Bill Act (P.L. 119-21) to ensure that rural providers are not required to navigate fragmented or conflicting federal expectations. These grants represent a significant, time-limited federal investment to help rural providers modernize care delivery and improve long-term sustainability amid meaningful resource constraints.

RHT program grantees are being asked to modernize care delivery under significant financial and workforce constraints. Any misalignment between AI policy guidance and grant requirements risks slowing adoption or diverting limited resources toward compliance rather than innovation. Clear harmonization, particularly around interoperable

data access, AI governance expectations and permissible clinical use cases, would enable rural providers to more effectively deploy AI tools supported by federal funding and mitigate post-grant technology cliffs. Harmonization is critical to enabling rural providers to turn investments in innovation into improved access, quality and efficiency of care.

V. AI-READY SUPPLY CHAINS REQUIRE INTEROPERABLE, STANDARDS-BASED DATA

Whether forecasting demand, detecting shortages, optimizing allocation or automating recall workflows, AI systems are powerful tools for improving healthcare supply chains. However, these tools are only as good as the data pipelines they consume. In healthcare supply chains today, non-standard identifiers, uneven barcode usage and siloed system designs degrade data quality, inhibit linkage across organizations and constrain model performance. Premier's latest [supply-chain insights](#) confirm that volatility and shortages persist, underscoring the need for real-time visibility and trustworthy data to power predictive and operational AI.

Global, open standards provide the most practical pathway to AI-grade data and bolster data quality and consistency. Within healthcare and adjacent sectors, [GS1 standards](#), including **GTIN** (products), **GLN** (locations), **GS1-128/2D barcodes** (lot/serial/expiry) and **EPCIS** (event-level data exchange), create a common language for identification, capture, and sharing that models can reliably learn from and act on. These standards are proven, technology-neutral, and already widely globally deployed across supply chains.

Current data gaps undermining AI deployment in healthcare supply chain

Label/identity ambiguity leads to corrupted features and model drift. When the same product or location is represented by different proprietary IDs across systems, AI pipelines struggle with entity resolution, leading to duplicate or misattributed records and skewed features (e.g., false stockouts, phantom inventory). Premier's experience shows that adopting GLN as a common location "Rosetta stone" and GTIN/UDI for products reduces mis-shipments and procurement errors -- exactly the kinds of noise that degrade model accuracy and stability.

Incomplete capture of lot/serial/expiry leads to gaps in risk and recall inference. AI-assisted recall triage and risk scoring depend on precise, machine-readable item attributes. GS1-128/2D barcodes and UDI (GTIN) encode those attributes for consistent ingestion into analytics and AI services, enabling faster, automated matching to inventories and patient records and preventing product expiry or counterfeit use and allow authentication and tracking of products throughout the supply chain.

Siloed event data leads to limited forecasting and anomaly detection. Predictive AI for shortages and lead times requires standardized, time-stamped events (shipping, receipt, aggregation, de-aggregation). EPCIS provides a common schema (already implemented across the global health care supply chain) and vocabulary for these supply-chain events, allowing models to learn from comparable signals across trading partners and technology platforms.

Uneven adoption means network effects never materialize for AI. Premier's supply chain resiliency findings indicate continued disruption and backorders, highlighting that individual adopters cannot unlock system-level AI benefits without broader network uptake. Federal alignment is needed to close the adoption gap and allow AI to generalize across sites and suppliers.

Premier's recommendations

Incentivizing the movement toward fully interoperable supply chain data is essential for both clinical and administrative functions in the healthcare sector. Federal leadership can uniquely address persistent barriers to supply chain data interoperability. Accordingly, Premier recommends that ASTP/ONC consider the following:

Embedding supply chain identifiers in ONC certification criteria and APIs. ASTP/ONC should ensure that certified health IT (including EHR-adjacent inventory/OR modules) can capture, store, and exchange GTIN/UDI and GLN, and can ingest GS1 barcodes/2D symbols at receipt and point of care. Premier recommends requiring these

identifiers to be accessible via developer APIs so AI services can consistently consume them across organizations. This addresses Premier's observation that systems too often "do not speak GTIN/UDI," limiting downstream analytics and AI.

Publishing cross-agency guidance endorsing open standards for AI data pipelines. ASTP/ONC coordinate a unified HHS position that GS1 identifiers (GTIN, GLN) and EPCIS event data constitute the recommended, non-proprietary foundation for supply-chain data used in AI/ML, reducing mapping overhead and error rates while improving explainability and auditability of AI outputs.

Advancing UDI/GTIN alignment for AI-assisted safety and quality. Premier urges ASTP/ONC to coordinate across HHS – particularly with the Food and Drug Administration (FDA) – to reinforce GTIN as the canonical product key in provider systems and item masters so AI agents can link product identity to lot/serial/expiry, claims and clinical context for safety analytics, recall automation and other clinically-relevant insights.

Normalizing GLN for facility sub-location identity. Premier encourages ASTP/ONC to work across HHS to promote GLN for standardized location master data used in supply-chain, administrative, and clinical flows. Normalized geographies and care settings improve AI feature engineering (e.g., demand signals by department) and reduce mis-routing that would otherwise confound model training.

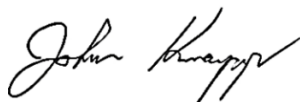
Requiring machine-readable recall/shortage communications. Premier urges ASTP/ONC to coordinate across the various HHS operating divisions to encourage or require inclusion of GTIN, GLN, and lot/serial in structured, machine-readable formats (e.g., EPCIS) so provider AI services can automate signal detection, inventory matching and patient impact assessment with minimal human reconciliation.

Interoperability in supply-chain data is foundational to national healthcare resilience, safety, and efficiency. Premier's experience underscores both the feasibility and the necessity of applying open, global standards such as GTIN, GLN, GS1 barcodes, and EPCIS to unify the healthcare supply chain. ASTP/ONC is uniquely positioned to drive this transformation by embedding data-standard expectations into certification, federal guidance and cross-agency coordination. With targeted leadership, HHS can help shift the system from fragmented and reactive to connected, transparent, and resilient, benefits patients and improving outcomes.

VI. CONCLUSION

Premier appreciates the opportunity to provide feedback on incentivizing innovation and AI adoption in healthcare. For questions, please contact Mason Ingram, Senior Director of Government Affairs, at Mason_Ingram@PremierInc.com.

Sincerely,



John Knapp
Vice President of Advocacy
Premier Inc.